Salofalk Klysmen Gebrauchsinformation (GI) für Malaysia
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PACKAGE INSERT

1. NAME OF THE MEDICINAL PRODUCT

Salofalk® 2g/30ml enemas
Salofalk® 4g/60ml enemas

Rectal suspension

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

(Salofalk 2g/30ml enemas)
One Salofalk 2g/30ml enema (= 30 g rectal suspension) contains 2 g mesalazine.

(Salofalk 4g/60ml enemas)
One Salofalk 4g/60ml enema (= 60 g rectal suspension) contains 4 g mesalazine.

Excipients with known effect
One Salofalk 2g/30ml enema contains 140.4 mg potassium metabisulphite and 30 mg sodium benzoate.

One Salofalk 4g/60ml enema contains 280.8 mg potassium metabisulphite and 60 mg sodium benzoate.

For a full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Rectal suspension
Very light tan to brown homogenous suspension, free from foreign matter

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

(Salofalk 2g/30ml enemas)
Mild and moderate attacks of ulcerative colitis (a chronic inflammatory disease of the large bowel) limited to the rectum and the sigmoid colon.

(Salofalk 4g/60ml enemas)
Acute attacks of ulcerative colitis (a chronic inflammatory disease of the large bowel)

4.2 Posology and method of administration

Posology

(Salofalk 2g/30ml enemas)
Adults and elderly
In patients with symptoms of acute inflammation, the content of two enema bottles (2 x 30 g suspension) is instilled in the rectum as an enema once daily at bedtime.

Children
There is little experience and only limited documentation for an effect in children.

If there are problems to retain the large quantity of liquid, the Salofalk 2g/30ml enemas may also be applicated in two doses, e.g. during the night (after a bowel movement which discharges the first dose) or early in the morning.

(Salofalk 4g/60ml enemas)
Adults and elderly
In patients with symptoms of acute inflammation, the content of one enema bottle (60 g suspension) is instilled in the rectum as an enema once daily at bedtime.

Children
There is little experience and only limited documentation for an effect in children.

General instructions for use
Salofalk 4g/60ml enemas are used once daily at bedtime.

Treatment with Salofalk enemas must be administered regularly and consistently, because only in this way can healing be successfully achieved.

The duration of use is determined by the physician.

Method of administration
Rectal use.
The best results are achieved if the bowel is emptied before administration of the Salofalk enemas.

Preparation:
- The bottle should be shaken for 30 seconds.
- Then the protective cap of the applicator is removed.
- The bottle should be held at the top and the bottom.

The correct position for administration is as follows:
- The patient should lie down on his/her left side with his/her left leg stretched out and right leg bent. This makes it easier for the rectal suspension to be administered and for the enema to be effective.

Administration of the rectal suspension:
- The tip of the applicator should be inserted deep into the rectum.
- The bottle should be tipped downwards slightly and then squeezed slowly.
- Once the bottle is empty, the applicator tip should be slowly withdrawn from the rectum.
- The patient should remain lying down in this position for at least 30 minutes to allow the contents of the enema to spread throughout the rectum.
- If possible, the rectal suspension should be allowed to exert its effects all night.

4.3 Contraindications

Salofalk enemas are contraindicated in patients with
- known hypersensitivity to the active substance, salicylates or to any of the excipients listed in section 6.1
- severe impairment of hepatic or renal function

4.4 Special warnings and precautions for use

Blood tests (differential blood count; liver function parameters like ALT or AST; serum creatinine) and urinary status (dip sticks) should be determined prior to and during treatment, at the discretion of the treating physician. As a guideline, follow-up tests are recommended 14 days after commencement of treatment, then a further two to three tests at intervals of 4 weeks.

If the findings are normal, follow-up tests should be carried out every 3 months. If additional symptoms occur, these tests should be performed immediately.

Caution is recommended in patients with impaired hepatic function.

Mesalazine should not be used in patients with impaired renal function. Mesalazine-induced renal toxicity should be considered, if renal function deteriorates during treatment. If this is the case, Salofalk enemas should be discontinued immediately.

Cases of nephrolithiasis have been reported with the use of mesalazine including stones with a 100% mesalazine content. It is recommended to ensure adequate fluid intake during treatment.

Mesalazine may produce red-brown urine discoloration after contact with sodium hypochlorite bleach (e.g. in toilets cleaned with sodium hypochlorite contained in certain bleaches).

Serious blood dyscrasias have been reported very rarely with mesalazine. Hematological investigations should be performed if patients suffer from unexplained haemorrhages, bruises, purpura, anaemia, fever or pharyngolaryngeal pain. Mesalazine should be discontinued in case of suspected or confirmed blood dyscrasia.

Cardiac hypersensitivity reactions (myocarditis, and pericarditis) induced by mesalazine have been rarely reported. Mesalazine should then be discontinued immediately.

Patients with pulmonary disease, in particular asthma, should be very carefully monitored during a course of treatment with mesalazine.

Photosensitivity
More severe reactions are reported in patients with pre-existing skin conditions such as atopic dermatitis and atopic eczema.

Severe cutaneous adverse reactions
Severe cutaneous adverse reactions (SCARs), including drug reaction with eosinophilia and systemic symptoms (DRESS), Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN), have been reported in association with mesalazine treatment. Mesalazine should be discontinued, at the first appearance of signs and symptoms of severe skin reactions, such as skin rash, mucosal lesions, or any other sign of hypersensitivity.

Idiopathic intracranial hypertension
Idiopathic intracranial hypertension (pseudotumor cerebri) has been reported in patients receiving mesalazine. Patients should be warned for signs and symptoms of idiopathic intracranial hypertension, including severe or recurrent headache, visual disturbances or tinnitus. If idiopathic intracranial hypertension occurs, discontinuation of mesalazine should be considered.

Patients with a history of adverse drug reactions to preparations containing sulphasalazine should be kept under close medical surveillance on commencement of a course of treatment with mesalazine. Should Salofalk enemas cause acute intolerance reactions such as abdominal cramps, acute abdominal pain, fever, severe headache and rash, therapy should be discontinued immediately.

Salofalk enemas contain potassium metabisulphite which may rarely cause severe hypersensitivity reactions and bronchospasm.

This medicinal product contains 30 mg sodium benzoate in each Salofalk 2g/30ml enema and 60 mg sodium benzoate in each Salofalk 4g/60ml enema. Sodium benzoate may cause local irritation.

4.5 Interaction with other medicinal products and other forms of interaction

Specific interaction studies have not been performed.

In patients who are concomitantly treated with azathioprine, 6-mercaptopurine or thioguanine, a possible increase in the myelosuppressive effects of azathioprine, 6-mercaptopurine or thioguanine should be taken into account.

There is weak evidence that mesalazine might decrease the anticoagulant effect of warfarin.

4.6 Fertility, pregnancy and lactation

Pregnancy
There are no adequate data from the use of mesalazine in pregnant women. However, data on a limited number of exposed pregnancies indicate no adverse effect of mesalazine on pregnancy or on the health of the fetus/newborn child. To date no other relevant epidemiologic data are available. In one single case after long-term use of a high dose of mesalazine (2-4 g, orally) during pregnancy, renal failure in a neonate was reported.

Animal studies on oral mesalazine do not indicate direct or indirect harmful effects with respect to pregnancy, embryonic/fetal development, parturition or postnatal development.

Salofalk enemas should only be used during pregnancy if the potential benefit outweighs the possible risk.

Breastfeeding
N-acetyl-5-aminosalicylic acid and to a lesser degree mesalazine are excreted in breast milk. Only limited experience with mesalazine during lactation in women is available to date. Hypersensitivity reactions such as diarrhoea in the infant cannot be excluded. Therefore, Salofalk enemas should only be used during breast-feeding if the potential benefit outweighs the possible risk. If the infant develops diarrhoea, the breastfeeding should be discontinued.

4.7 Effects on ability to drive and use machines

Mesalazine has no or negligible influence on the ability to drive or use machines.

4.8 Undesirable effects

See table at the end of this package leaflet.

4.9 Overdose

There are rare data on overdosage (e.g. intended suicide with high oral doses of mesalazine), which do not indicate renal or hepatic toxicity. There is no specific antidote and treatment is symptomatic and supportive.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Intestinal anti-inflammatory agents; aminosalicylic acid and similar agents
ATC code: A07EC02

Mechanism of action
The mechanism of the anti-inflammatory action is unknown. The results of in vitro studies indicate that inhibition of lipoxygenase may play a role.
Effects on prostaglandin concentrations in the intestinal mucosa have also been demonstrated. Mesalazine (5-Aminosalicylic acid / 5-ASA) may also function as a radical scavenger of reactive oxygen compounds.

Pharmacodynamic effects
On reaching the intestinal lumen, rectally administered mesalazine has largely local effects on the intestinal mucosa and submucosal tissue.

5.2 Pharmacokinetic Properties

General considerations of mesalazine

Absorption
Mesalazine absorption is highest in proximal gut regions and lowest in distal gut areas.

Biotransformation
Mesalazine is metabolised both pre-systemically by the intestinal mucosa and the liver to the pharmacologically inactive N-acetyl-5-aminosalicylic acid (N-Ac-5-ASA).
The acetylation seems to be independent of the acetylator phenotype of the patient. Some acetylation also occurs through the action of colonic bacteria. Protein binding of mesalazine and N-Ac-5-ASA is 43% and 78%, respectively.

Elimination
Mesalazine and its metabolite N-Ac-5-ASA are eliminated via the faeces (major part), renally (varies between 20% and 50%, dependent on kind of application, pharmaceutical

preparation and route of mesalazine release, respectively), and biliary (minor part). Renal excretion predominantly occurs as N-Ac-5-ASA. About 1% of total orally administered mesalazine dose is excreted into the breast milk mainly as N-Ac-5-ASA.

<u>Salofalk 2g/30ml enemas specific</u>

Distribution
An imaging study in patients with mild-to-moderate acute ulcerative colitis showed that the rectal suspension at the start of treatment and at remission after 12 weeks is distributed mainly in the rectum and sigmoid colon.

Absorption and elimination
No specific pharmacological studies on Salofalk 2g/30ml enemas are available.

In a study on Salofalk 4g/60ml enemas in ulcerative colitis patients in remission, peak plasma concentrations of 0.92 µg/ml 5-ASA and 1.62 µg/ml N-Ac-5-ASA were achieved after approximately 11-12 hours under steady-state conditions. The elimination rate was approximately 13% (45-hour value), with most (approximately 85%) being eliminated in the form of the metabolite, N-Ac-5-ASA.
The steady-state plasma concentrations of 5-ASA and N-Ac-5-ASA in children with chronic inflammatory bowel disease under treatment with Salofalk 2g/30ml enemas were 0.2-1.0 µg/ml and 0.4-2.0 µg/ml respectively.

<u>Salofalk 4g/60ml enemas specific</u>

Distribution
An imaging study in patients with mild-to-moderate acute ulcerative colitis showed that the rectal suspension at the start of treatment and at remission after 12 weeks is distributed mainly in the rectum and sigmoid colon and to a lesser extent in the descending colon.

Absorption and elimination
In a study in ulcerative colitis patients in remission, peak plasma concentrations of 0.92 µg/ml 5-ASA and 1.62 µg/ml N-Ac-5-ASA were achieved after approximately 11-12 hours under steady-state conditions. The elimination rate was approximately 13% (45-hour value), with most (approximately 85%) being eliminated in the form of the metabolite, N-Ac-5-ASA.
The steady-state plasma concentrations of 5-ASA and N-Ac-5-ASA in children with chronic inflammatory bowel disease under treatment with Salofalk 4g/60ml enemas were 0.5-2.8 µg/ml and 0.9-4.1 µg/ml respectively.

5.3 Preclinical safety data

Preclinical data reveal no special hazard for humans based on conventional studies of safety pharmacology, genotoxicity, carcinogenicity (rat) or toxicity to reproduction.

Kidney toxicity (renal papillary necrosis and epithelial damage in the proximal tubule (pars convoluta) or the whole nephron) has been seen in repeat-dose toxicity studies with high oral doses of mesalazine. The clinical relevance of this finding is unknown.

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

(Salofalk 2g/30ml enemas)
Carbomer 35 000; potassium acetate; potassium metabisulphite (max. 0.14 g, equivalent to max. 0.08 g SO_2); sodium benzoate; disodium edetate; water, purified; xanthan gum

(Salofalk 4g/60ml enemas)
Carbomer 35 000; potassium acetate; potassium metabisulphite (max. 0.28 g, equivalent to max. 0.16 g SO_2); sodium benzoate; disodium edetate; water, purified; xanthan gum

6.2 Incompatibilities

Not applicable.

6.3 Shelf life

2 years.

6.4 Special precautions for storage

Store below 30 °C and protect from light.

6.5 Nature and contents of container

(Salofalk 2g/30ml enemas)
<u>Container</u>: Round, white, concertina-shaped LDPE bottle with a red protective LDPE cap

(Salofalk 4g/60ml enemas)
<u>Container</u>: Round, white, concertina-shaped LDPE bottle with a green protective LDPE cap

<u>Pack sizes</u>: Packs containing 7 enemas

6.6 Special precautions for disposal and other handling

No special requirements.

7. MANUFACTURER / PRODUCT REGISTRATION HOLDER

Manufactured by
Losan Pharma GmbH
Otto-Hahn-Straße 13
79395 Neuenburg
Germany

For:

DR. FALK PHARMA GmbH
Leinenweberstr. 5
79108 Freiburg im Breisgau
Germany

Product Registration Holder:
DCH Auriga (Malaysia) Sdn Bhd (719-V),
Lot 6, Persiaran Perusahaan,
Seksyen 23, 40300 Shah Alam,
Selangor

8. PRODUCT REGISTRATION NUMBER

MAL19912973ACZ

9. DATE OF FIRST AUTHORISATION /RENEWAL OF THE AUTHORISATION

Date of first authorisation: 14.08.1991
Date of last renewal: 05.09.2025

10. DATE OF REVISION OF THE TEXT

May 2025

11. GENERAL CLASSIFICATION FOR SUPPLY

Medicinal product subject to medical prescription

Undesirable effects

The following undesirable effects have been observed after administration of mesalazine

System organ class	*Frequency according to MedDRA convention*			
	Common *(≥ 1/100 to<1/10)*	*Rare* *(≥ 1/10,000; <1/1,000)*	*Very rare* *(< 1/10,000)*	*Not known* *(cannot be estimated from the available data)*
Blood and lymphatic system disorders			Altered blood counts (aplastic anaemia, agranulocytosis, pancytopenia, neutropenia, leukopenia, thrombocytopenia)	
Nervous system disorders		Headache, dizziness	Peripheral neuropathy	Idiopathic intracranial hypertension (see section 4.4)
Cardiac disorders		Myocarditis, pericarditis		
Respiratory, thoracic and mediastinal disorders			Allergic and fibrotic lung reactions (including dyspnoea, cough, bronchospasm, alveolitis, pulmonary eosinophilia, lung infiltration, pneumonitis)	
Gastrointestinal disorders		Abdominal pain, diarrhoea, flatulence, nausea, vomiting	Acute pancreatitis	
Renal and urinary disorders			Impairment of renal function including acute and chronic interstitial nephritis and renal insufficiency	Nephrolithiasis*
Skin and subcutaneous tissue disorders	Rash, pruritus	Photosensitivity	Alopecia	Drug reaction with eosinophilia and systemic symptoms (DRESS), Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN)
Musculoskeletal and connective tissue disorders			Myalgia, arthralgia	
Immune system disorders			Hypersensitivity reactions such as allergic exanthema, drug fever, lupus erythematosus syndrome, pancolitis	
Hepatobiliary disorders			Changes in liver function parameters (increase in transaminases and parameters of cholestasis), hepatitis, cholestatic hepatitis	
Reproductive system disorders			Oligospermia (reversible)	

* see section 4.4 for further information

Severe cutaneous adverse reactions (SCARs), including drug reaction with eosinophilia and systemic symptoms (DRESS), Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN), have been reported in association with mesalazine treatment (see section 4.4).